

King Fahad Central Hospital

EMS–Hospital Stroke System of Care Collaboration Agreement

Aligned with AHA/ASA Mission: Lifeline Stroke and Joint Commission Stroke Center Certification requirements for prehospital coordination

Hospital / Stroke Center: _____	EMS Agency: _____
Certification Level: ASR / PSC / TSC / CSC	Service Area/Region: _____
Effective Date: _____	Review/Renewal Date: _____

1. Purpose

This agreement establishes a coordinated stroke system of care between the EMS Agency and the Hospital/Stroke Center, defining prehospital stroke recognition, pre-arrival notification, destination/triage protocols, data sharing, and joint quality improvement processes, consistent with AHA/ASA Mission: Lifeline Stroke and applicable stroke center certification requirements for prehospital coordination.

2. Prehospital Stroke Recognition & Screening

- ☐ EMS personnel use a standardized, validated prehospital stroke screening tool on all patients with suspected stroke symptoms (e.g., Cincinnati Prehospital Stroke Scale, FAST-ED, or agency-approved equivalent)
- ☐ EMS personnel use a standardized large-vessel-occlusion (LVO) severity screen where regionally adopted (e.g., RACE, LAMS, VAN, or agency-approved equivalent) to inform destination decisions
- ☐ EMS personnel determine and document Last Known Well (LKW) time, symptom onset/discovery time, and obtain fingerstick glucose per protocol
- ☐ Hospital provides initial and ongoing education to EMS personnel on stroke recognition tools and their correct application (see Section 6)

3. Pre-Arrival Notification Protocol

- ☐ EMS activates pre-arrival stroke notification to the receiving hospital for all patients with a positive prehospital stroke screen, as early as feasible and no later than ___ minutes prior to estimated arrival
- ☐ Pre-arrival notification includes, at minimum: LKW time, stroke screen result (including LVO screen result if used), estimated time of arrival, vital signs, blood glucose, and pertinent history (anticoagulant use, recent surgery, etc.)
- ☐ Hospital activates stroke alert/code stroke team immediately upon receiving pre-arrival notification, prior to patient arrival, per hospital protocol
- ☐ Receiving hospital unit is prepared for direct-to-CT transport per hospital's Acute Stroke CT Imaging Protocol

4. Destination & Triage / Bypass Protocol

Reconcile this section with the applicable state/regional EMS destination plan; regional protocol governs where it conflicts with this template.

- ☐ Suspected stroke patients transported to the nearest hospital capable of providing appropriate stroke-level care within the regionally defined transport time standard
- ☐ Patients with a positive LVO screen preferentially transported (bypass) to a Thrombectomy-Capable or Comprehensive Stroke Center when within the regionally approved additional transport time allowance, per regional EMS destination protocol
- ☐ Inter-facility transfer coordination: hospital and EMS agency maintain an agreed process for timely transfer of patients requiring a higher level of stroke care (e.g., thrombectomy) not available at the initial receiving facility
- ☐ Destination decisions documented by EMS per agency protocol, including rationale for any deviation from standard destination routing

5. Patient Care Report / Data Sharing

- ☐ EMS agency provides the patient care report (PCR) to the receiving hospital at time of patient handoff or per agreed electronic data-sharing mechanism
- ☐ Hospital provides feedback to EMS agency on patient outcomes and stroke-system performance metrics for quality improvement purposes, consistent with applicable state peer-review/quality-assurance protections and HIPAA
- ☐ Data shared between parties limited to the minimum necessary for stroke system of care quality improvement, per HIPAA and applicable state law
- ☐ Both parties designate a data-sharing point of contact responsible for coordinating information exchange:
EMS — _____ ; Hospital — _____

6. Joint Education & Training

- ☐ Hospital stroke program provides periodic education sessions to EMS personnel on stroke recognition, screening tools, and system updates (frequency: _____)
- ☐ Joint case reviews conducted periodically to review stroke system performance, including notable cases, discordant screens, and false activations, in a non-punitive quality improvement format
- ☐ EMS agency participates in hospital-led stroke system of care committee meetings or provides a designated liaison
- ☐ Joint simulation drills or system-of-care exercises conducted periodically as agreed by both parties

7. Performance Metrics Tracked Jointly

Metric	Description
EMS response and on-scene time	Time from dispatch to patient contact; on-scene time for suspected stroke calls
LKW documentation rate	Percentage of suspected stroke runs with LKW time documented
Pre-arrival notification rate	Percentage of confirmed stroke patients for whom pre-arrival notification was given
Prehospital stroke screen accuracy	Concordance between EMS screen result and final hospital diagnosis
Symptom-onset-to-door time	Time from LKW/onset to hospital arrival
Scene-to-needle / scene-to-puncture time (system metric)	Total system time from EMS patient contact to treatment, tracked in aggregate for system-of-care improvement

8. Governance & Oversight

- ☐ A joint EMS–Hospital Stroke System of Care oversight group (or existing regional stroke system committee) meets at defined intervals (e.g., quarterly) to review performance data and quality issues
- ☐ Hospital Stroke Medical Director and EMS Medical Director (or designees) serve as co-leads for clinical protocol alignment
- ☐ Disagreements regarding destination protocol interpretation or performance issues addressed first through the oversight group before escalation

9. Term, Renewal & Termination

- ☐ This agreement is effective as of the Effective Date above and remains in effect for [specify term, e.g., 1–2 years], subject to periodic review
- ☐ Agreement reviewed and renewed per the Review/Renewal Date above, or automatically renews unless either party provides written notice of intent not to renew at least [specify] days prior
- ☐ Either party may terminate this agreement with [specify] days' written notice to the other party
- ☐ This agreement will be reviewed and updated as needed to remain consistent with current regional EMS protocols, state regulations, and stroke center certification requirements

10. General Provisions

- ☐ Nothing in this agreement supersedes the EMS agency's medical direction, state EMS regulations, or the regional EMS destination plan, which govern where any conflict exists
- ☐ Data exchanged under this agreement is subject to HIPAA and applicable state confidentiality/peer-review protections; specific data-sharing mechanisms and any required business associate agreement to be addressed in a separate document as needed, per institutional legal counsel review
- ☐ This agreement does not create liability obligations beyond those otherwise established by law; liability terms to be reviewed and specified by legal counsel for both parties
- ☐ Amendments to this agreement require written agreement of both parties

Signatures

Hospital Stroke Medical Director: _____ Date: _____

Hospital Administration Representative: _____ Date: _____

EMS Medical Director: _____ Date: _____

EMS Agency Administration Representative: _____ Date: _____

References: AHA/ASA Mission: Lifeline Stroke prehospital and system-of-care recommendations; 2019 AHA/ASA Guidelines for the Early Management of Patients With Acute Ischemic Stroke and subsequent focused updates; The Joint Commission Primary/Comprehensive/Thrombectomy-Capable/Acute Stroke Ready certification requirements for EMS coordination; applicable state EMS destination plan and regulations. This template must be reviewed by legal counsel and reconciled with regional EMS protocols before execution, and re-verified against the current certifying-body manual at the time of institutional adoption.